

Policy on the Established Discharge Process

«Hospital Name»

Document control

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Prepared by	Medical Superintendent Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

Statement of intent

«Hospital Name» has an established discharge process: planned in consultation with the patient and/or family, coordinated among departments and agencies including medico-legal and absconded cases, governed by written LAMA guidance, issuing a discharge summary to every patient, adhering to planned discharge, expanding access through domiciliary visits where applicable, and monitoring discharge time for continual improvement.

1. Purpose

This policy explains how «Hospital Name» manages the discharge process: planning discharge with the patient and family, coordinating discharge across departments and agencies, governing discharge against medical advice, issuing a discharge summary to every leaving patient, adhering to planned discharge, providing domiciliary visits where applicable, and monitoring and improving discharge time.

The content of the discharge summary is covered in the hospital's other policies.

2. Scope

This policy applies to treating doctors, nursing staff, accounts and other departments involved in discharge, and all patients and families at «Hospital Name».

3. Policy standards

The discharge process is planned in consultation with the patient and/or family and coordinated among all departments and agencies involved, including medico-legal and absconded cases. Written guidance governs the discharge of patients leaving against medical advice, and a discharge summary is given to all patients leaving the organisation.

The organisation adheres to planned discharge, expands access to health practices through domiciliary visits wherever applicable, and monitors discharge time, setting appropriate benchmarks for continual improvement.

4. Non-negotiable rules

- Do not discharge a patient without planning the process in consultation with the patient and/or family.
- Do not discharge a medico-legal case without informing the police, or handle an absconded case outside the organisation's coordination discipline.
- Do not allow a patient to leave against medical advice without following the written LAMA guidance and recording the declaration in the required format.
- Do not let any patient leave the organisation — including one leaving against medical advice or in case of death — without issuing a discharge or death summary.
- Do not claim to offer a domiciliary visit service that falls outside the scope of services defined in the hospital's service directory.

5. What we do

5.1 Plan discharge in consultation with the patient and family

The discharge process is planned in consultation with the patient and/or family. The treating doctor determines readiness for discharge during regular reassessments and discusses the plan with the patient and family, covering expected timing, continued care needs, and who will receive the discharge summary.

5.2 Coordinate discharge among departments and agencies

The discharge process is coordinated among all departments and agencies involved, including accounts, so that discharge papers are completed within time. For medico-legal cases, the police are informed about the discharge. Absconded-patient workflows follow the same coordination discipline when the patient is found or the case is otherwise closed.

5.3 Govern discharge against medical advice

Written guidance governs the discharge of patients leaving against medical advice (LAMA). The treating doctor explains the consequences of leaving to the patient or attendant. The written guidance addresses reasons for LAMA so the organisation can take corrective and/or preventive action where patterns appear. The patient/attendant declaration is

recorded in a proper format. LAMA does not cancel the duty to give a discharge summary and reports.

5.4 Issue a discharge summary to every leaving patient

A discharge summary is given to all patients leaving the organisation, including patients leaving against medical advice. In case of death, a death summary is given to the next of kin or relatives. The organisation hands over the discharge summary and reports to the patient or attendant in all cases and retains a copy in the medical record.

5.5 Adhere to planned discharge

Discharge is planned at least 24 hours in advance. Planning includes preparation of the draft discharge summary, refund of medications where applicable, patient education on continued care, and identification of special needs — for example special equipment, devices, or transportation. Unplanned discharges are minimised.

5.6 Provide domiciliary visits where applicable

Where the organisation's scope includes domiciliary visits, trained healthcare workers visit patients at their residences to deliver care and support — for example for chronic conditions, fall prevention, memory care, nutrition, exercise, or palliative catheter care. Visits are scheduled at discharge planning when applicable and recorded. A hospital whose scope does not include domiciliary visits records this as not applicable and does not fabricate the service.

5.7 Monitor discharge time and improve it continually

The organisation monitors discharge time, setting appropriate benchmarks and making continual improvement. Discharge time is defined from when the treating doctor declares the patient fit for discharge to when the patient vacates the bed. Benchmarks may differ by payer mix. Delays are monitored, reasons identified, and improvement activities performed.

6. Stop-work authority

Not applicable. This standard has no stop-work authority. Gaps in discharge planning, coordination, LAMA guidance, or discharge-summary issuance are addressed through the hospital's quality monitoring and CAPA process (Section 8), not through an immediate stop-work trigger. The patient's right to refuse treatment and leave against medical advice is a patient right, not a stop-work matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for the discharge process across all departments and agencies.
Treating Doctor	Determines discharge readiness; explains LAMA consequences; ensures discharge

	summary is prepared and handed over.
Nursing Superintendent	Coordinates discharge planning and domiciliary visit scheduling with nursing staff.
Accounts/Administration	Completes discharge papers within the defined coordination timeframe.
Quality Coordinator	Audits this policy; monitors discharge time and holds LAMA and CAPA records.

8. Quality monitoring

Monitoring area	What is monitored
Discharge planning	Reassessment notes stating readiness; discussion records with patient/family.
Departmental coordination	Documented discharge procedure; MLC police intimation records; absconded-case coordination records.
LAMA	Written LAMA guidance; patient/attendant declarations; record of consequences explained.
Discharge summary issuance	Copy of discharge summary retained in the medical record for every leaver sampled; death summary acknowledgement.
Planned discharge	Expected discharge noted in advance; draft summary and special-needs preparation; unplanned discharge log with reasons.
Domiciliary visits	Scope statement; visit schedule and notes for eligible patients.
Discharge-time monitoring	Discharge-time log from fit-for-discharge to bed vacated; delay analysis with improvement actions.

9. Training and staff acknowledgement

All treating doctors, nursing staff, and departments involved in discharge shall be trained on the discharge process, LAMA guidance, and discharge-summary requirements in this policy, at induction and periodically.

I have read the Policy on the Established Discharge Process of «Hospital Name». I will follow the processes described.

10. Distribution

This policy shall be available to all clinical staff, accounts and administration, department heads, and the Quality Coordinator.

11. Abbreviations

Abbreviation	Meaning
CAPA	Corrective and Preventive Action
LAMA	Leaving/Left Against Medical Advice
MLC	Medico-Legal Case
NABH	National Accreditation Board for Hospitals and Healthcare Providers

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means the objective element is starred and requires documented evidence.

Objective Element	Level	Traceability to this policy
AAC.12.a	Commitment	Sections 3 and 5.1 address discharge planning in consultation with the patient and family.
AAC.12.b	Commitment*	Sections 3 and 5.2 address coordination of discharge among departments and agencies.
AAC.12.c	Commitment*	Sections 3 and 5.3 address written guidance for discharge against medical advice.
AAC.12.d	Commitment	Sections 3 and 5.4 address issuing a discharge summary to every leaving patient.
AAC.12.e	Commitment	Sections 3 and 5.5 address adherence to planned discharge.
AAC.12.f	Commitment	Sections 3 and 5.6 address domiciliary visits where applicable.
AAC.12.g	Commitment	Sections 3 and 5.7 address monitoring and improving discharge time.

13. Required records/evidence checklist

Discharge planning — AAC.12.a (Commitment)

- Reassessment note stating readiness for discharge.
- Record of discussion with patient and/or family about the discharge plan.

Departmental coordination — AAC.12.b (Commitment*)

- Documented discharge procedure covering department coordination including accounts.
- MLC discharge file with police intimation record.
- Absconded-case coordination record where applicable.

LAMA guidance — AAC.12.c (Commitment*)

- Current LAMA written guidance.
- Patient/attendant LAMA declaration in the required format.
- Record that the treating doctor explained consequences.

Discharge summary issuance — AAC.12.d (Commitment)

- Copy of discharge summary retained in the medical record for every leaver sampled.
- LAMA cases with summary handed over; death summary acknowledgement by next of kin.

Planned discharge — AAC.12.e (Commitment)

- Expected discharge noted at least 24 hours in advance for planned cases.
- Draft discharge summary and special-needs preparation before discharge.
- Log of unplanned discharges with reasons and minimisation actions.

Domiciliary visits — AAC.12.f (Commitment)

- Scope statement or written note of when domiciliary visits apply.
- Domiciliary visit schedule and visit notes for eligible patients.

Discharge-time monitoring — AAC.12.g (Commitment)

- Defined discharge-time benchmarks (by payer mix where used).
- Discharge-time log; delay analysis with continual-improvement actions.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. AAC.12.

Disclaimer

This policy reorganises the supplied AAC.12 objective-element wording and Guidebook interpretation into plain-language guidance for «Hospital Name». It does not replace the NABH Accreditation Standards for Hospitals, 6th Edition, or the Guidebook, which remain the authoritative source in case of any conflict.

This policy is intended for accreditation and internal governance purposes and is not a statement of, or substitute for, compliance with any specific statute; «Hospital Name»

remains separately responsible for meeting all applicable statutory and regulatory requirements, including medico-legal reporting obligations.

This policy shall be reviewed at least once every year, or earlier if NABH standards change.

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